جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : تيسير سيد محمد عبدالفتاح
المحافظة : القاهره **الحي / المدينة :** التبين **العنوان :** التبين-القاهره
رقم البطاقة : 27904170102426 **رقم التليفون :** 01020774816 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 17-04-1979 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: تيسير سيد محمد عبدالفتاح
National ID: 27904170102426

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
تيسير
سيد محمد عبد الفتاح
مساكن الحديد والصلب ب ٤٨ ح٣
التبين - القاهره
٢٧٩٠٤١٧٠١٠٢٤٢٦
JJ6944160


٢٧٩٠٤١٧٠١٠٢٤٢٦
حاصله على دبلوم معهد فنى تجارى
٢٠٢٢/٠٥
أنثى
مسلمة
متزوجة
سامى عبدالعاطى سيد عبدالفتاح
البطاقة سارية حتى ٢٠٢٩/٠٥/٢٤

USTAVE/
OUSSY
INTERNATIONAL

Patient Name	: الأستاذه / تيسير سيد محمد عبدالفتاح	Request No.	: 13006732
Patient No.	: 27904170102426	Request Date	: 15/12/2022
Path No.	: 3771/22	Verification Date	: 19/12/2022
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 43 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Bilateral malignant breast masses.

Nature of the specimen: Tru cut biopsies collected at 15/12/2022 at 11.24 am.

Gross Examination:
1- Right breast mass: Multiple grayish white tissue cores up to 2cm long, totally submitted.
2- Left breast mass: Multiple grayish white tissue cores up to 1.5cm long, totally submitted.

Microscopic examination:
1- Right breast mass:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II- III.
* DCIS: No
* Angioinvasion: Couldn't be detected in the received biopsy
* Perineural invasion : No.

2- Left breast mass:
* Histological type: Mixed Invasive lobular and ductal carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II.
* Insitu component: Minimal.
* Angioinvasion: Couldn't be detected in the received biopsy
* Perineural invasion : No.

Diagnosis:
Right breast mass; Tru cut biopsy:
- Invasive duct carcinoma G II- III.
- For IPT (ER, PR, Her2 and Ki67).

Verified by :
DR.Menna Nabil

CAP ACCREDITED
غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: تيسير سيد محمد **MR. :** 550	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

Metastatic breast cancer (MBC) to bone was noted on aromatase inhibitor (AI) therapy; the patient presented for administration of Femara and was prescribed supportive treatment for three months, along with Zoladex 10.8 mg. A CT of the chest, abdomen, and pelvis (CT CAP) was ordered and a bone scan was pending.

The patient continued on Femara for three months in combination with Zometa and Zoladex for management of bone metastases.

Femara was continued with supportive therapy while awaiting a new CT CAP and bone scan.

The patient has bilateral breast cancer with metastatic spread to bone. Right breast: clinical T2, N1, M0, invasive ductal carcinoma (IDC) grade 2-3, estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, HER2-negative, Ki-67 12 %. Left breast: clinical T2, N1, M0, invasive lobular carcinoma (ILC), ER 8/8, PR 8/8, HER2-negative, Ki-67 25 %. Initial bone scan in March 2023 showed a single lesion; subsequent imaging in January 2023 demonstrated widespread osseous disease. The patient has been on Zoladex and AI therapy since January 2023, with bone scans remaining negative since October 2023 and a free bone scan documented in June 2026. Recent CT (January 2026) showed only diffuse axial and appendicular sclerotic lesions; a new CT for AI assessment is pending.

Comparison of the CT performed on 1 July shows a stationary course of axial and appendicular sclerotic lesions, and the bone scan remains negative as of 28 June 2026. The patient continues AI therapy, Zoladex, and Zometa after laboratory results (CA125 4.4 on 21 June, serum creatinine 0.4). Femara for three months was obtained from the pharmacy.

The patient received palliative radiation therapy to two lumbar vertebrae and cervical vertebrae, with the cervical treatment administered in July 2025.

The patient's metastatic breast cancer pain regimen includes tramadol 300 mg daily, gabapentin 900 mg daily, and paracetamol 1500 mg daily, which are reported as well controlled.

Metastatic breast cancer to bone is being managed with Femara, Zoladex, and supportive care for three months; the next follow-up visit is scheduled for 2 December 2026.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : هنا اسامه عبد الرحيم عبد البر

يعتمد ,

USTAVE/
OUSSY
INTERNATIONAL

Patient Name	: الأستاذه / تيسير سيد محمد عبدالفتاح	Request No.	: 13006732
Patient No.	: 27904170102426	Request Date	: 15/12/2022
Path No.	: 3771/22	Verification Date	: 19/12/2022
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 43 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Left breast mass; Tru cut biopsy:

* Mixed Invasive lobular and ductal carcinoma Grade II with minimal Insitu component.
* For IPT (ER, PR,Her2 and Ki67).

Prof. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

CAP ACCREDITED

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JSTAVE/
OUSSY
NATIONAL

Patient Name	: الأستاذه / تيسير سيد محمد عبدالفتاح	Request No.	: 13006732
Patient No.	: 27904170102426	Request Date	: 15/12/2022
Path No.	: 3771/22	Verification Date	: 25/12/2022
Sex	: Female	Financial Category	: جوستاف روسي - تنقة الدولة
Age	: 43 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Bilateral malignant breast masses.

Nature of the specimen: Tru cut biopsies collected at 15/12/2022 at 11.24 am.

Gross Examination:
1- Right breast mass: Multiple grayish white tissue cores up to 2cm long, totally submitted.
- Left breast mass: Multiple grayish white tissue cores up to 1.5cm long, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- Right breast mass:
Histological type: Invasive duct carcinoma .
Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II- III.
DCIS: No
Angioinvasion: Couldn't be detected in the received biopsy
Perineural invasion : No.
IPT (Positive control showed appropriate reactivity) :
ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8)
HER2: No membrane reaction (score 0)
Ki 67: 18% nuclear reaction of tumor cells

- Left breast mass:
Histological type: Mixed Invasive lobular and ductal carcinoma .
Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II.

Verified by :
Dr.Amin Ahmed

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USTAVE/
OUSSY
INATIONAL

Patient Name	الأستاذه / تيسير سيد محمد عبدالفتاح	Request No.	13006732
Patient No.	27904170102426	Request Date	15/12/2022
Path No.	3771/22	Verification Date	25/12/2022
Sex	Female	Financial Category	جوستاف روسي - نفقة الدولة
Age	43 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Insitu component: Minimal.
Angioinvasion: Couldn't be detected in the received biopsy
Perineural invasion : No.
IPT (Positive control showed appropriate reactivity) :
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 75% of tumor cells (Allred score 8/8)
HER2: Weak incomplete membrane reaction of tumer cells <10% (score +1)
Ki 67: 25% nuclear reaction of tumor cells

Diagnosis:
) Right breast mass; Tru cut biopsy:
Invasive duct carcinoma G II- III.
PT:
ER: Positive (score 8/8).
PR: Positive (score 8/8).
HER2: Negative (score 0).
Ki67: Positive in 18% of tumor cells.

) Left breast mass; Tru cut biopsy:
Mixed Invasive lobular and ductal carcinoma Grade II with minimal Insitu component.
PT:
ER: Positive (score 8/8).
PR: Positive (score 8/8).
HER2: Negative (score +1).
Ki67: Positive in 25% of tumor cells.

Prof. Lubna Omar.
Dr. Menna Nabil.

Verified by :
Dr.Amin Ahmed

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

مستشفى الملك فهد الجامعي
King Fahd University Hospital

MDT FORM 17/ 1 /2023 G

Name	[illegible]	MRN	[illegible]
		ID	[illegible]
Age	43	Phone	[illegible]
Menopausal status		Marital status	
NRS SCORE		Family history	1st degree (Sister)
BMI		REASON OF MDT	Bx Re
CO morbidity			

BX: Rt - Mixed lobular & ductal Ca.B.
Lt - Mix. Ca. B. III.

Clinical Data: Ca breast & ? Bone. Mets (Bilateral)

ER: 8/8 | PR: 8/8 | HER 2: -ve | KI67: 18% Rt 23% Lt

Sonomammography:
12/2022 - Multiple UOQ Masses in both breasts (Rt) BIRADS 5)

Mets Workup:
CT S, - widespread Skeletal osseous Lesions Likely Mets

R / L



TNM

Recommendations:
Bilateral Ca breast (neglected) Gen almost 1 year & ? Bone. Mets
Refer Med Onc & Assessment.

MDT OUTCOME & PLAN
[illegible]

SIGNATURE					
[illegible]	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name : taisir sayed mohamed	**Date : 26-01-2026 03:06**
Branch : Portable	**Medical No. : GRI-26455**
Ref.Physician Prof : cap+c	**Ins.Company :**

CT chest, abdomen and pelvis with contrast

Previous Exams:

- This is the baseline metastatic work-up, non-comparative study.

FINDINGS:

- **Chest:**
  - *Clear both pulmonary parenchymal lung fields with no focal mass lesions, nodules or consolidative patches*
  - The trachea is central in position.
  - No mediastinal mass lesions, normal enhancement of the mediastinal vessels.
  - No suspicious mediastinal or hilar lymph nodes.
  - Normal pleural thickening with no pleural collections identified.
  - Normal cardiac size. No pericardial effusion.
- **Abdomen:**
  - The liver shows normal size and density with no enhancing focal hepatic lesions.
  - Cholecystectomy clips are noted with multiple metallic structures related to the ileal loops ? for appendectomy … for clinical correlation.
  - Normal size and density for the spleen with no enhancing focal mass lesions.
  - The pancreas shows normal size with no ductal dilatation or enhancing focal mass lesions.
  - Normal appearance of the supra renal glands bilaterally.
  - Both kidneys show normal size with no suspicious mass lesions. Intact parenchymal thickness, no renal stones or back pressure changes,
  - Normal ureters with no hydronephrotic changes.
  - Scans through the pelvis show no mass lesions. IUCD is insitu.
  - No suspicious enlarged abdominal or pelvic lymph nodes. No ascites.
- *Scans through the bone shows diffuse axial and appendicular skeleton osseous sclerotic nodules.*

Opinion: LIMITED STUDY DUE TO LACK OF MEDICAL DATA OR PAST HISTORY

- **Unremarkable chest study with contrast.**
- **Unremarkable abdominal and pelvic study with contrast.**
- **Scans through the bone shows diffuse axial and appendicular skeleton osseous sclerotic nodules … mostly deposits for bone scan assessment .**

Thanks for referral;

Dr. Reham Mohamed, MS., FRCR.

Patient Name: Taisir Sayed Mohamed Abd Al Fattah	Date : 28/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID: 68/2023	Pat. ID: 26455

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case of breast carcinomas, compared to prior study dated 12/5/2025 ; the current study revealed:

1. **Chest:**
    - both breast are out of examination field
    - Still no sizable pulmonary nodules.
    - Few enlarged bilateral axillary L. Ns.
    - Few enlarged mediastinal lymph nodes, the largest at prevascular space 6mm.
    - No evidence of pleural effusion, pleural thickening or masses seen.
    - Average cardiac size. No pericardial effusion detected.
    - Normal CT appearance of the thoracic aorta and the great vessels.
2. **Abdomen and pelvis:**
    - The liver is average in size with homogenous low CT texture. No definite focal lesions or contour irregularities.
    - Patent homogenously enhancing portal vein.
    - No intra-hepatic or extra-hepatic biliary radicles dilatation.
    - Clips of previous cholecystectomy.
    - Normal size & parenchymal texture of the spleen with no focal lesions seen.
    - No pathological abdominal or pelvic lymph nodes.
    - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
    - Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
    - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
    - Normal appearance of the other pelvic organs. IUD is seen in uterine cavity.
    - No abdominal or pelvic masses or collections.
    - Still noted Right iliac fossa mesenteric metallic foreign bodies are noted.

Patient Name: Talsir Sayed Mohamed Abd Al Fattah	Date : 28/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 46 Year(s)
Medical ID: 68/2023	Pat. ID: 26455

3. **The scanned skeleton:**

- Stable widespread axial and appendicular skeleton sclerotic foci and areas.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- **Follow up status still showing rather stationary course since last study.**

Dr. Reham Khedr , M.D.

Not Approved

NASSER INSTITUTE
ONCOLOGY CENTER

NUCLEAR MEDICINE UNIT

Patient Name: Taiseer Sayed Mohamed | Date: 13/3/2023

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

- Active osseous lesion is noted at Lt. 5th rib and less active osseous lesions are noted at Rt. 7th rib and Rt. side of DV 8.
- Otherwise, No definite evidence of hot or cold spots that would account for metastatic bone lesions
- Both kidneys are normally seen.

Conclusion:

Positive bone scan for active osseous lesions as described above.

Thanks for reference

Nuclear Medicine Unit

Dr.Sherein Mahdy , MD
Dr.Mohamed AlaaEldin